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Robert C. Hockett, Ph.D.
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110 East 59th Street
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Dear Dr. Hockett:

Thank you very much for the privilege of reading this manuscript. I return it to you herewith and also enclose a copy of a recent article that has some interesting and apparently contradictory findings, namely that the VST returns to normal within a short time after experimental ligation of a coronary artery.

I am still very much disturbed with the clinical implication of this manuscript. As I wrote previously, the audience and the reader is conditioned to accept the results without recognizing that the experiments possibly lack clinical relevance. Thus,

(1) The authors point out that the Framingham study found that sudden death from coronary artery disease was four times as high in the heavy smoker as is the non-smoker. The authors also point out that the most likely cause for sudden death is ventricular fibrillation.

What the authors fail to state is that neither the Framingham group nor any other group found that sudden death from coronary artery disease occurs predominantly while smoking or sooner after smoking. If this were so, there would be little difficulty in proving cause and effect. On the contrary, the temporal relation between smoking and sudden death has never been documented. Indeed all evidence, at least within the hospital and within the Coronary Care Unit, indicates that ventricular fibrillation with or without sudden death occurs without immediate antecedent smoking. The experimental temporal relation therefore has no counter part in clinical medicine. I do believe that these points should be discussed and stressed in this article.

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